



InterQual® Level of Care Criteria

Inpatient Rehabilitation Criteria Review Process

Introduction

InterQual® Level of Care Criteria support determining the appropriateness of admission, continued stay, and discharge destinations for rehab patients. The Acute Adult Inpatient Rehabilitation Criteria and Subacute Inpatient Rehabilitation Criteria are for adult patients 18 years of age and older. The Pediatric Inpatient Rehabilitation Criteria are for children younger than 18 years of age.

Important: The criteria reflect clinical interpretations and analyses and cannot alone either resolve medical ambiguities of particular situations or provide the sole basis for definitive decisions. The criteria are intended solely for use as screening guidelines with respect to the medical appropriateness of healthcare services and not for final clinical or payment determinations concerning the type or level of medical care provided, or proposed to be provided, to a patient.

Note: Detailed notes and literature references provide the clinical basis for decisions. Less commonly, there are key clinical questions where evidence is limited or lacks consensus and may reflect topics which are not conducive to formal study. In these situations, standards of care are used to support the development of criteria which reflect the highest quality available literature and best practice. Identification and validation of standards of care is the product of a peer review process involving multiple clinicians with diverse expertise in varied relevant practice and geographic settings who assist with the translation of standards of care into evidence-based criteria.

InterQual® content contains references to patient sex or gender. Depending on the context, these references may refer to either genotype or phenotype. At the individual patient level, a variety of factors, including but not limited to gender identity and gender affirmation via surgery or hormonal treatment, may affect the applicability of some InterQual Criteria. This occurs most often with genetic testing and some procedures that require the presence of specific anatomy. For the purpose of these criteria, references to Male and Female are based on sex assigned at birth, unless otherwise defined. InterQual users should carefully consider patient genotype and anatomy when appropriate.

Informational notes

Informational notes provide information regarding best clinical practice, new clinical knowledge, explanations of criteria rationale, definitions of medical terminology, current literature references, and Medicare policy information relevant for a review of a Medicare beneficiary. A note icon indicates one or more notes are associated with a criteria point.

To view notes, click a note icon.

How to conduct a medical review

Inpatient Rehabilitation Criteria include four types of reviews: Preadmission, Admission, Continued Stay, and Discharge. Each type of review uses criteria components to determine the appropriateness of the level of care. There are three components:

- **Severity of Illness (SI):** Used to determine the severity of the patient's illness.
- **Intensity of Service (IS):** Represents monitoring and therapeutic services that can be administered at the specified level of care.
- **Discharge Screens (DS):** Used to determine whether the patient has reached a level of stability or independence appropriate for a safe discharge or transfer from the current level of care.

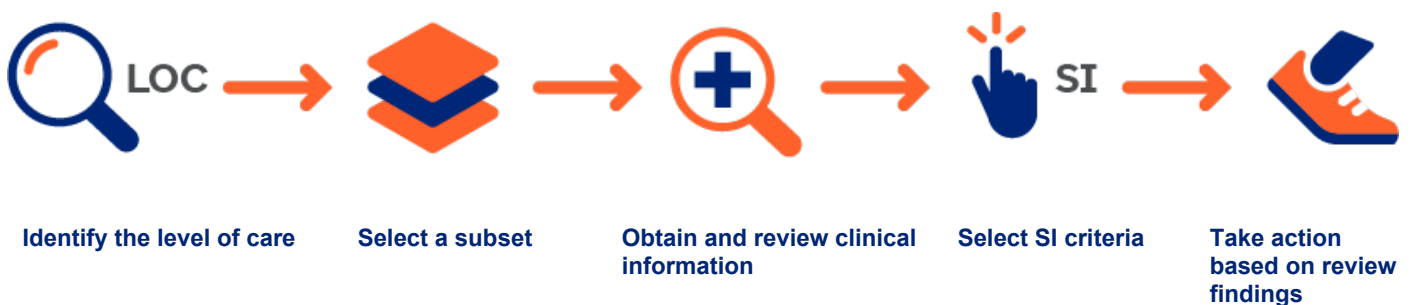
The following sections explain the process for conducting each type of review. As you conduct a review, observe the following guidelines:

- Review all notes attached to criteria subsets, rules, and criteria points.
- You may select as many criteria as the rules allow, or as specified by organizational policy for documentation purposes, as long as the minimum number of criteria has been met. For example, when the rule displays as "≥ One," you can select one or more underlying criteria points. When the rule displays as "One," you should select only one criterion.
- Criteria that state "at baseline," "> baseline," or "< baseline" refer to the patient's pre-illness or new baseline status.

Preadmission review

Conduct a Preadmission review to determine the appropriateness of an admission prior to a planned admission or transfer to a level of care. A Preadmission review uses the Severity of Illness (SI) criteria (Refer to page 7 for more information about Severity of Illness).

To conduct a Preadmission review, follow these steps:



1. Identify the level of care based on the patient's current or proposed level. Observe these guidelines:
 - When a facility's name (for example, Transitional Care Unit) does not match the InterQual Criteria subset titles, refer to the subset note located on the title page of a specific subset. The minimum requirements for monitoring and interventions generally provided at the specific level of care will be noted.
 - When a patient is located at a level of care that is different from the assigned level of care, you should use the criteria set aligned with the level of care assignment. For example, suppose a patient is in an acute rehab bed but is assigned subacute rehab. Use Subacute rehab criteria for the review.
2. Select the appropriate subset based on the patient's predominant presenting inpatient rehabilitation needs.

3. Obtain and review patient specific clinical information (for example, history, physical, laboratory, imaging, progress notes, and medical practitioner orders).
4. To apply the SI rule, select the SI criteria based on the patient's clinical findings and inpatient rehabilitation needs, making sure to meet all the rules for time of onset and number of criteria.

As you conduct the review, observe the following guideline:

- Criteria that state “within normal limits (WNL),” “within acceptable limits,” or “at baseline” refer to when a patient returns to his or her personal baseline.

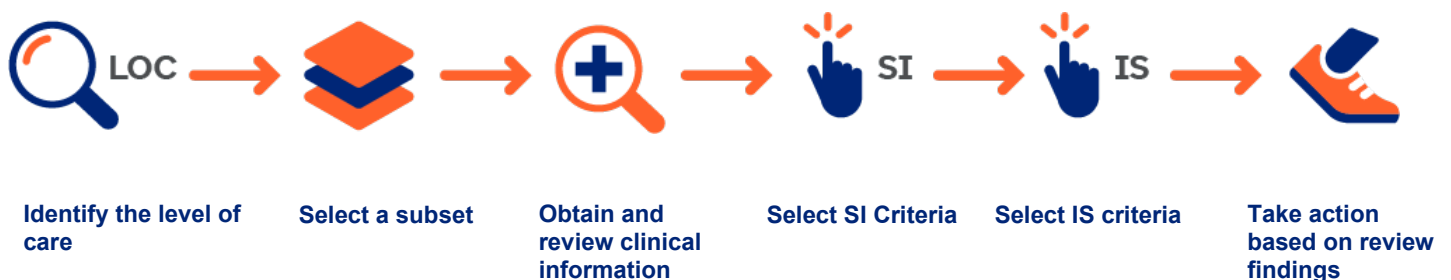
5. Take action, as follows:

Finding	Action
Preadmission rule met	Approve planned admission.
Preadmission rule not met	• Contact the attending medical practitioner for additional information to verify the need for admission to the Acute or Subacute Inpatient Rehabilitation level of care. • If the additional information satisfies the preadmission rule, approve the planned admission. • If the additional information does not satisfy the preadmission rule, refer for Secondary review.

Admission review

Conduct an Admission review when a patient is admitted to a level of care. The Admission review determines the appropriateness of that level of care. Apply the Severity of Illness (SI) criteria and Intensity of Service (IS) criteria derived from the first 4 days of admission.

To conduct an Admission review, follow these steps:



1. Identify the level of care based on the patient’s current or proposed level. Observe these guidelines:
 - When a facility’s name (for example, Transitional Care Unit) does not match the InterQual Criteria subset titles, refer to the subset note located on the title page of a specific subset. The minimum requirements for monitoring and interventions generally provided at the specific level of care will be noted.
 - When a patient is located at a level of care that is different from the assigned level of care, you should use the criteria set aligned with the level of care assignment. For example, suppose a patient is in an acute rehab bed but is assigned subacute rehab. Use Subacute rehab criteria for the review.
2. Select the appropriate subset based on the patient’s predominant presenting inpatient rehabilitation needs and clinical findings.

3. Obtain and review patient-specific clinical information (for example, history, physical, laboratory, imaging, progress notes, and medical practitioner orders).
4. To apply the SI rule, select criteria based on the patient's inpatient rehabilitation needs and clinical findings, making sure to meet all the rules for time of onset and number of criteria.

As you conduct the review, observe the following guidelines:

- Criteria that state “within normal limits (WNL),” “within acceptable limits,” or “at baseline” refer to when a patient returns to his or her personal baseline.
5. To apply the IS rule, expand the Admission Review criteria section. Select criteria based on the prescribed therapy, treatments, or interventions from the same criteria subset that you used to select SI, making sure to meet all the rules for duration and number of criteria.
 - When there is a range of days (for example, $\leq 2d$) associated with an IS criterion, you may approve up to the time frame, eliminating the need for weekly or daily review. The end point “ $\leq 2d$ ” indicates that the criteria point may be applied for no more than two days.
 - The Discharge Screens may be used to validate that the patient is not clinically stable for transfer or discharge before the end of the time frame.
 6. Take action, as follows:

Finding	Action
SI and IS rules met	• Approve admission to level of care. • Schedule Continued Stay review.
SI or IS rule not met	• Obtain additional information from the attending medical practitioner or other caregivers. • If additional information does not meet the corresponding SI or IS, discuss alternate levels of care with the attending medical practitioner. • Facilitate transfer if the attending medical practitioner agrees with an alternate level of care. • Refer for Secondary review if the attending medical practitioner does not agree with alternate level of care.

Continued Stay review

Conduct a Continued Stay review to determine the appropriateness of continued stay at the current level of care. Apply the Intensity of Service (IS) criteria to the review.

Important: A Continued Stay review should be conducted at least weekly. Though, the review frequency may vary based on organizational policy. Each time you conduct a Continued Stay review, evaluate the case since the last review to ensure the Intensity of Service (IS) has been met **daily**.

To conduct a Continued Stay review, follow these steps:



1. Select the same criteria subset that was used for the Admission review, with the following exceptions:
 - The patient has been transferred to a lower level of care. In this case, select the appropriate criteria subset based on the patient's clinical information.
 - The patient has been transferred to a higher level of care. In this case, conduct an Admission review, applying both SI and IS to determine if admission to the higher level is clinically appropriate.
 - The patient remains at the current level of care, but the medical condition has changed. In this case, you can use a different subset within that level of care. Apply only IS criteria.
2. Obtain and review patient-specific clinical information (for example, medical practitioner, nursing, therapy, and interdisciplinary team progress notes, medical practitioner orders, medication and treatment records).
3. To apply the IS rule, expand the Continued Stay section. Select a responder type based on the prescribed therapy, treatments, or interventions, making sure to meet all the rules for duration and number of criteria. Responder types include:
 - **Responder:** Criteria that indicate the patient is appropriate for discharge or transfer. Selection of these criteria “do not meet” for continued stay and are denoted by the symbol .
 - **Partial responder:** Criteria that indicate the patient is appropriate for continued stay.

As you conduct the review, observe the following guidelines:

- Criteria that state “within normal limits (WNL),” “within acceptable limits,” or “at baseline” refer to when a patient returns to his or her personal baseline.
 - When there is a range of days (for example, $\leq 2d$) associated with an IS criterion, you may approve up to the time frame, eliminating the need for weekly or daily review. The end point “ $\leq 2d$ ” indicates that the criteria point may be applied for no more than two days.
 - The Discharge Screens may be used to validate that the patient is not clinically stable for transfer or discharge before the end of the time frame.
4. Take action, as follows:

Finding	Action
IS Partial responder met	• Approve level of care. • Schedule next Continued Stay review.
IS Responder met	• Prepare for discharge or transfer. • Review discharge screens to determine the most appropriate post-acute level of care.
IS Partial responder and Responder not met	• Obtain additional information from the attending medical practitioner or other caregivers. • If IS still not met, conduct discharge review.

Discharge review

Conduct a Discharge review when criteria for continued stay are not met. A Discharge review assists you in determining the next appropriate level of care within the facility (a transfer to another unit) or in determining discharge from the facility. A Discharge review uses the Discharge Screens (DS) criteria.

Important: The word “Discharge” in Discharge Screens refers to discharge (transfer) from one level to another level of care, not necessarily discharge from the facility.

To conduct a Discharge review, follow these steps:



Select the same subset used for Admission review or Continued Stay review

Select DS criteria

Take action based on review findings

1. Select the same criteria subset that was used for the Admission or Continued Stay review.
2. Apply the DS rule for the appropriate level of care.
3. Take action, as follows.

Review reason	Finding	Action
IS Responder met or IS Partial responder not met or IS Responder not met	DS met	• If discharge is scheduled, no action required. • If discharge is not scheduled: – Contact the attending medical practitioner to discuss the discharge plan and alternate level of care options. –Facilitate discharge or transfer if the attending medical practitioner agrees. –Refer for Secondary review if the attending medical practitioner does not agree with the alternate level of care.
IS Responder met or IS Partial responder not met or IS Responder not met	DS not met	Refer for Secondary Review if the attending medical practitioner does not agree with the alternate level of care.

Documenting variances

When Discharge Screens are met and an alternate level of care is appropriate but unavailable you should:

1. Indicate the reason the patient has not been transferred.
2. Assign a level of care that represents the level of care that would be appropriate for the patient if it had been available.
3. Document the number of days (referred to as variance days) used at a specific level of care when a less intensive, less costly level is appropriate.
4. Discuss the case with a secondary reviewer and document the review decision.

InterQual Level of Care components

Level of Care components screen the appropriateness of admission to, continued stay at, and discharge from care. The InterQual Inpatient Rehabilitation Criteria cover adult and pediatric populations. The Adult Criteria are organized into Acute and Subacute subsets. The Pediatric subset covers Acute Inpatient Rehabilitation. There are three components:

- Severity of Illness (SI)
- Intensity of Service (IS)
- Discharge Screens (DS)

Severity of Illness

Severity of Illness (SI) criteria consist of objective clinical indicators.

- The SI rule requires that all SI criteria be met.
- The time requirements vary based on the level of care. If there is a time requirement, it is associated with SI criteria. For example:
 - Illness, injury, surgery within the last 30 days (Adult and Pediatric Inpatient Rehabilitation)
- SI criteria address the following:
 - Clinical features of the patient's illness appropriate for the specific level of inpatient rehabilitation care.
 - Patient's ability and willingness to benefit from a comprehensive acute or subacute inpatient rehabilitation program.
 - Unavailability of services at a lower level of care.

Intensity of Service

Intensity of Service (IS) criteria consist of therapeutic, diagnostic, and monitoring services, singularly or in combination, that can be administered at a specific level of care.

- The IS rule requires that Partial responder criteria be met.
 - **Responder:** Criteria that indicate the patient is appropriate for discharge. Selection of these criteria “do not meet” for continued stay and are denoted by a symbol .
 - **Partial Responder:** Criteria that indicate the patient is appropriate for continued stay.
- The IS time requirement is “At Least Daily.”
- IS criteria include requirements for duration of therapy per day and the number of hours or days per week. For example:
 - At least 2 disciplines $\geq 3\text{h/d} \geq 15\text{h/wk}$ (Adult Inpatient Rehabilitation)
 - At least 2 disciplines $\geq 2\text{-}3\text{h/d} \geq 5\text{d/wk}$ (Subacute Rehabilitation)
- Some IS criteria are associated with a duration of time, which are intended to allow you to approve up to the number of days indicated. The days are based on a calendar day, which starts at 12:01 a.m. regardless of the time of admission. However, the exception to this would be admissions in the evening (for example, after 6 p.m.). In this case, day one would not begin until the next day.

Note: Regulatory or contractual agreements may dictate other specifics concerning when the “new day” begins. For example, “Medical instability (new onset) and decreased participation in therapy $< 3\text{h/d}$ for $\leq 3\text{d}$.” If the patient was able to participate in therapy on a given day and developed a new

medical instability later that evening, then the first day of counting the decrease in therapy would start the next morning.

Discharge Screens

When Responder criteria are met, use the Discharge Screens (DS) to assist in determining the most appropriate post-acute level of care. The DS are a resource tool and not criteria. Referring to the DS at the initiation of discharge planning is recommended. DS include ongoing service needs for post-acute levels of care.

The DS are organized from the least to most intensive level of care.

There is no time requirement for DS.

Note: For a guide to planning a safe and effective transition to a post-acute level of care, refer to the InterQual® Transition Plan tool. (See “InterQual® Transition Plan tool” below.)

Utilization Benchmarks

Benchmark length of stay (LOS) targets are included in Inpatient Rehabilitation Criteria for most conditions. The InterQual benchmark length of stay values are derived from a set of deidentified claims data, representing millions of patient discharges and have been statistically validated. Values represent geometric mean length of stay for a specific condition and are based on ICD claims including co-morbidities. For conditions with more than one ICD code (e.g., cerebral vascular accident (CVA)), the LOS represents a weighted average based on the number of claims for each included ICD code.

The LOS value provides a benchmark that is not intended to serve as a treatment limit or substitute for clinical judgment but can support medical directors, utilization leaders, and other hospital and health plan professionals understand normal ranges for current practice.

InterQual® Transition Plan tool

The Transition Plan tool assists you in planning for a patient’s safe transition to the most appropriate post-acute level of care. You are encouraged to begin using the Transition Plan tool at the time of admission. The Transition Plan:

- Is NOT a required part of the review process
- Outlines interventions necessary to ensure continuity of quality care
- Identifies patients who are at high risk for readmission
- Provides a framework for identifying discharge needs

Secondary review

Secondary review determines the appropriateness of the current or proposed level of care when it is not supported by criteria on primary review. Organizational policy should dictate the extent to which secondary review is performed to render a review outcome.

When is a secondary review appropriate?

- Review criteria are not met

- Criteria are met and there is a concern about the level of care based on the complexity of the patient's condition
- There are questions about the quality of care
- As identified by the organization
- There is a discrepancy in review outcomes between payers and providers

What questions does a secondary review answer?

- Does the patient require admission or continued stay at the current level of care?
- Are there considerations beyond the primary review criteria to be considered?
 - Does the patient require this level of care?
 - What are the treatment options?
 - Is there a quality-of-care question?
 - Should a specialist evaluate this case?

Who is responsible for completing a secondary review?

A secondary review may be completed by a supervisor, medical practitioner, subject matter expert or designated clinician. A medical practitioner is **not** required to perform a secondary review; however, some regulatory requirements specify that only a physician can issue a final non-determination (denial) decision.

Secondary review steps

1. The secondary reviewer determines medical necessity based on review of the medical record; discussions with members of the interdisciplinary team (e.g., nursing staff, the discharge planner, therapists, and the attending medical practitioner); and clinical knowledge and judgment.

Note: The secondary reviewer is not required to but may choose to apply InterQual criteria when completing a secondary review. Best practice would include reviewing the results of the primary review to ensure internal agreement in criteria application.

To determine medical necessity, it may be appropriate for the secondary reviewer to consider other factors in the determination, such as:

- Age
- Co-morbid conditions (e.g., medical, substance use, and psychiatric disorders)
- Risk factors for readmission
- Presence of a caregiver, life stressors, or safe home environment
- Social determinants of health (such as low health literacy and racial disparities)
- Treatment response and patient engagement
- History of non-compliance and health outcomes
- Other organization or patient-specific factors

Tip: A primary reviewer can assist the secondary review process by including the outcome of the InterQual review, as well as documentation that could support these additional factors.

2. Determine the review outcome:
 - If the secondary reviewer agrees with the existing level of care, approve the level of care.
 - If the secondary reviewer does not agree with the existing level of care, they discuss the alternate level of care options for this patient with the attending medical practitioner.

- If the attending medical practitioner agrees with the secondary reviewer, facilitate the transfer to the alternate setting or level of care, if available.
 - If the attending medical practitioner does not agree with the secondary reviewer, initiate action as indicated by organizational policy.
3. Document the review outcome.

Reference materials

Reference materials are provided with the criteria and should be used to assist in the correct interpretation of the criteria.

- **Abbreviations and Symbols List** — Defines acronyms, abbreviations, and symbols used in the criteria.
- **Bibliography** — Provides references cited in the clinical content.
- **Clinical Revisions** — Provide details of changes to InterQual Clinical Criteria.
- **Drug List** — Categorizes drug names and classes mentioned within the criteria.
- **Index** — Lists conditions and/or diagnoses and is designed to guide the user to the criteria subset where a specific condition or diagnosis may be found.
- **InterQual® Transition Plan Tool** — Assists in planning for a safe transition to the most appropriate post-acute level of care.
- **Length of Stay** — Provides length of stay reference data based on diagnosis.

Additional resources

InterQual Resource Center

The [InterQual® Resource Center](#) is a central location for the most up to date InterQual clinical documentation and resources.

The Resource Center provides access to:

- **What's New** — Includes release highlights for each of the InterQual® Criteria modules included in a content release.
- **Clinical Revisions** — Includes the clinical revisions for the current InterQual Criteria year and two years prior.
- **Clinical Resources** — Includes information such as the Knowledge Articles, Known Issues List, and more.
- **Webinars** — Includes the Increase Your IQ educational webinar recordings.
- **Additional Resources** — Includes access to resources such as Customer Care Hub, the InterQual® Learning Source, and Download Connect.

Customer Care Hub

[Customer Care Hub](#) is a web portal that enables you to submit, update, and view support case details and status.

To obtain a user ID and password, from the Customer Care Hub Welcome page, select Request Account.